National Cancer Control Programme (NCCP) 00318 · clinician draft

Pemetrexed + Carboplatin

Which treatment this guide covers This guide covers pemetrexed and carboplatin chemotherapy. It does not include an immunotherapy medicine. If nivolumab, pembrolizumab or another medicine is also prescribed, obtain its separate regimen, consent, monitoring and emergency instructions. Mesothelioma and non-small-cell lung cancer (NSCLC) have different aims and evidence; ask which indication is on your prescription.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pemetrexed + Carboplatin · chemotherapy day

Active medicines: pemetrexed + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V8 row Reference course only: reconcile with pemetrexed rash prophylaxis and aprepitant interaction into one unambiguous prescription. Do not add both courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.

Additional phase-specific support

- High emetic risk driven by carboplatin. Use dedicated AUC≥4 carboplatin NCIS V8 row; no routine olanzapine in that row.
- Reconcile pemetrexed rash and antiemetic dexamethasone into one prescription.
- Folic acid daily and B12 every three cycles; continue folate21days beyond the last pemetrexed.
- Verify renal function for AUC5 carboplatin dosing; no routine cisplatin hydration schedule is implied.
- An additional nivolumab or other drug needs its own regimen and support plan.
- When platinum stops or another phase begins, review the antiemetic plan; do not carry the entire high-risk course into pemetrexed-only maintenance automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00318 v6 · pNCCP00318 pp1–7; NCIS V8 Table1 · NCCP NCIS Medical Oncology antiemetics V8, Table 1

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required